



## ENTRY PROTOCOL

### Touchstone CAMHS

|                       |                  |
|-----------------------|------------------|
| <b>Scope (Staff):</b> | All staff        |
| <b>Scope (Area):</b>  | Touchstone CAMHS |

#### Child Safe Organisation Statement of Commitment

CAHS commits to being a child safe organisation by applying the National Principles for Child Safe Organisations. This is a commitment to a strong culture supported by robust policies and procedures to reduce the likelihood of harm to children and young people.

This protocol is currently under review to be incorporated into the relevant Model of Care.  
The content remains current throughout this review process.

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## Aim

This protocol describes the entry process to Touchstone Child and Adolescent Mental Health Service (CAMHS) Day Program, including clear information about which children are eligible for the service and the process for facilitating access to alternative care for individuals not accepted by the service.

## Risk

- A delay in the provision of appropriate care for a highly vulnerable population of children.
- An inability to gain entry to the appropriate service.

## Background

This entry protocol aligns with the CAMHS Access policy. It describes entry as required by the National Standards for Mental Health Services (Standard 10.3).

Touchstone CAMHS consists of a multidisciplinary team that provides consultation, assessment and a treatment program for children presenting with self-harm and associated difficulties such as impulsivity, affect regulation and interpersonal relationships.

The target age range is 12 to 17 years of age. Touchstone CAMHS operates on the principles of a therapeutic community and integrates Mentalization Based Therapy (MBT), an evidence-based psychological treatment for Borderline Personality Disorder, in the context of individual, group, family therapy and creative therapies. Pharmacological treatment is managed by the Head of Service Consultant Psychiatrist and utilized when required as an adjunct to treatment.

## Definitions

**Access:** Ability of consumers or potential consumers to obtain required or available services when needed and within an appropriate time.

**Entry:** The process provided by the mental health service which assists the consumer and their carers to make contact with the mental health service and receive appropriate assistance.

**Child:** Infant, child, adolescent or young person under the age of 18.

**Parent:** Any parent, legal guardian, responsible family member, or other identified carer of the child.

## Key points

- The safety of the children and their families is paramount and barriers to access to Touchstone CAMHS will be minimised.
- Interventions will be based on evidence-based practice and encourage a shift in emphasis from problem focus to a perspective of building child and family uniqueness, strengths, resilience and recovery.

- Working with children and their families in recovery-focussed ways. In working together with children, families and support networks, Touchstone CAMHS will support them to become decision makers in their own care, implementing the principles of recovery-oriented child and adolescent mental health practice.
- Recovery-oriented practice supports and recognises the following:
  - The uniqueness of the individual;
  - Attitudes and rights;
  - Dignity and respect; and
  - Partnerships and communication.
- Key principles include the following:
  - Provides a holistic framework that informs all contact with children and families;
  - Builds and enhances strength, resilience and social well-being;
  - Supports children to reach their expected developmental trajectory and full potential;
  - Is underpinned by the premise that children do recover from mental health problems;
  - Engages well with all areas of the child and family's life, including relationships, education, vocation and leisure;
  - Informs the recovery plan that is regularly reviewed by the child, family and multidisciplinary team;
  - Adopts a culture of enquiry empowering children and families to become experts by experience; and
  - Use the ethos of a therapeutic community which includes safety, communication, participation, peer support and positive risk-taking.

## Process

### Service Information

#### Referral and Assessment

The Touchstone CAMHS Day Program aims to offer intervention to complex presentations in children with severe self-harm.

Indicators of the complexity of their presentation which are evaluated at intake point are the following:

- The child must have attempted individual therapy (and family work where available) in a less intensive Tier 3 mental health service or a specialised Tier 4

service, from which they have not benefited (not only because of lack of engagement);

- The child is a frequent user of acute services (either emergency department presentations or inpatient admissions);
- High impact of symptoms such as those affecting school, social contexts and home, causing obvious deterioration;
- Chronicity of presentation; and
- Severe attachment disruption.

Referrals can be made by CAMHS that are based in the Perth metropolitan area. In addition, when practical for families to commit to the Day Program despite residing outside of the Perth metropolitan area, referrals from WACHS can be accepted as well.

Case management will continue to be held by the referring service while the child takes part in the introductory group, however once the child commences the Day Program, the case management role will be taken over by Touchstone CAMHS.

Referrals can be received by fax and/or email and the referrer is notified via email by Touchstone of the receipt of the referral. The referral provides information on family and developmental background, history of presenting symptoms and past treatments indicating effective and ineffective interventions, attachment pattern and family function, academic and social function as well as expectations and obstacles of an intensive treatment approach.

If further information is required, the triage officer will contact the referrer and/or the child/family/carers directly, prior to presenting it to the Multidisciplinary Team (MDT) for discussion.

The MDT meets weekly to discuss new referrals and assess whether they meet the criteria for assessment. Following this meeting, if the referral is accepted for the program, a letter is provided to the referrer informing them of the outcome. Two members of the MDT will conduct assessment of the child and their family. Touchstone administration will make contact with the child/family/carers and offer four assessment appointments. If the family is unable to attend the appointments offered to them, new appointment times will be negotiated with them.

The findings of assessment are discussed with the MDT in a weekly meeting and a decision is made regarding entry into the Day Program.

The outcome of the assessment is summarized in a written report that is shared and discussed face to face with the child, their family and if possible the referrer and then sent to all parties.

The assessments for the Touchstone Day Program inclusion involve:

- Four different sessions where the child and family are seen both separately and together with the aim to clarify the features of the presentation, to assess the level of risk, to elicit the child and parents capacity to mentalize, and to identify

the domains of mentalizing that they struggle with so that a working formulation can be developed and then worked on during the program. The assessments also aim to understand more about the child's relationships, including significant family history, attachments and any notable trauma. In the spirit of encouraging explicit collaboration, the family is presented with the child and family formulation in the last assessment session so that they have an opportunity to clarify and amend any information that does not fit with their understanding.

- Completion of self-report and clinician-administered research measures to contribute to the overall clinical understanding of the formulation of both family and carers. Measures include mentalization capacity, self-harm behaviours, Borderline Personality Disorder features, attachment styles and more.
- Ability to understand (cognitively and emotionally) the purpose of the program, what it involves and its challenges and to be able to use mentalization based therapy.
- Decision to attend the program being not overly influenced by the referrer, family or others but the child being autonomous in intent.
- Willingness to commit to the program by acknowledging expectations and the role that children, family and Touchstone are going to play in collaboration.

Any factors that will affect the child's capacity to engage and participate will be assessed. This may include:

- Active severe psychotic symptoms;
- Malnourishment impacting on cognitive and emotional processing or requiring inpatient treatment;
- Current CPFS involvement due to recent trauma or ongoing instability in family environment; or
- Significant developmental delay.

In cases where the child does not meet the entry criteria, or where the child and family choose not to opt-in to the program or are assessed as being unable to make use of an MBT intervention, the family, child and referrer will be notified of the outcome and alternative suggestions will be developed collaboratively with the family and the referrer.

Also see [Appendix 1 Day Program Care Pathway Flowchart](#) and the Touchstone Model of Care (in development) for further information on the operational processes, procedures and guidelines for the service.

### Guiding Principles in Specific Circumstances

The Touchstone Day Program:

- Ensures that mechanisms are in place to enable access for children within the developmental demographic criteria (age 12 to 17 years).

- Does not allow families to self-present a child to the clinic. Referrals must come through Tier 3 Community CAMHS regardless of the child's residential address.
- Identifies and outreaches to marginalised groups, including LGBTIQ+ children. Collaboration with other relevant specialist services particular to marginalised groups ensures that all needs of patients in these population groups are addressed.
- Will work with interpreters and other culturally-relevant supports with culturally and linguistically diverse families, to ensure that a culturally sensitive and appropriate service is offered.
- Recognises the special needs of Aboriginal families and accesses Aboriginal Liaison Officers and Aboriginal Mental Health Workers, when necessary, to assist in supporting families and children of Aboriginal background.
- Ensures that the service is accessible to any child that needs the service (that is, any child meeting entry criteria whose needs are not better serviced elsewhere). Waiting times for assessments are kept to a minimum and thorough assessment ensures the best fit of treatment for the individual and family.
- Provides and maintains least restrictive practice, a developmentally, age-appropriate and culturally-appropriate safe environment for all children and their families/carers in line with the therapeutic community principles of inclusion, participation, containment, safety and positive risk taking.
- Upholds children's rights as identified in the [UN Convention on the Rights of the Child](#) and in the [CAMHS Patient Rights under the Mental Health Act Policy](#).
- Encourages families and children to explore appropriate alternative treatment providers when considering their treatment options alongside the recommendations made by Touchstone, both before accessing the program and at any stage during the treatment journey. Touchstone liaises with other service providers, including schools to ensure comprehensive care and communication.
- Provides informed consent and care for access and/or entry for families with diverse language needs.

## Access to Touchstone Day Program in a Range of Settings

Touchstone CAMHS is an integrated service within the wider Specialised CAMHS. The primary goal is to tailor treatment to the needs of the adolescent population of the Perth metropolitan area and non-metropolitan area, when access to Perth is not a barrier.

Care provided in other settings includes:

- Social and educational group or individual outing;
- Sessions in different community CAMHS under the Shared Care policy;
- Public transport training;

- Linking with community-based organisations and activities; and
- Liaising with the Education Department as required.

### **Access to Acute Services: 24 hours per day, 7 days a week**

Children attend the Touchstone Day Program during business hours on Mondays, Wednesdays and Fridays. They may also receive phone or in-person support on non-therapy days (Tuesdays and Thursdays).

Assessment of new referrals and introductory group sessions are conducted generally on Tuesdays and Thursdays.

Where urgent care is required, this is accessible via emergency department presentation, 24 hours a day, 7 days a week, as outlined in the individual Risk Assessment and Management Plan (RAMP) and Worries and Actions Support Plan (WASP). Patients, carers and families are informed of emergency telephone supports available after hours for urgent support and advice, as well as the potential physical and psychiatric symptoms they should be alert to that would require emergency intervention.

Should a child require 24-hour care and an inpatient admission be indicated Touchstone staff will attempt direct negotiation with CAMHS Ward 5A if under 16 or the local Youth Unit, if over 16. If however the crisis emerges after hours, admission should be sought via presentation to the emergency department. In all cases, Touchstone representatives will liaise with services, during business hours, to facilitate a safe return to therapy at Touchstone.

### **Physical Access**

Families/carers are provided with a map of the site and availability of parking. Touchstone has capacity to assist with transport to or from the program in exceptional circumstances, via public transport.

### **Consumer Privacy and Confidentiality**

Touchstone ensures that the right to consumer privacy and confidentiality is upheld where safety is not compromised. Confidentiality should not be a barrier to effective collaboration with other professionals/services. Patients and families admitted to the service are regularly informed of their right to privacy and confidentiality, and the limits of this. Patient records are maintained with utmost confidentiality and security in accordance with the [CAMHS Medical Record Management Procedure](#).

### **Engagement with Children, Families and Stakeholders**

Touchstone engages children and families in the review, design and evaluation of entry protocols and in an ongoing way on the review, design and evaluation of service culture, functioning and delivery at Touchstone.

Throughout the initial assessment process Touchstone works in a collaborative way with children and families in developing a shared understanding of their difficulties.



Children and families have been and continue to be involved in the design and evaluation of Touchstone consumer information materials. For example:

- The Touchstone Brochure; and
- The design and evaluation of the Touchstone poster which elaborates, in a youth-friendly manner, entry details in line with Touchstone's entry criteria.

Prior to entry into the program, Touchstone conducts a pre-start meeting with children, families and the support of their case manager in order to develop and plan goals for treatment in a collaborative manner.

Throughout the program the parents and children are regularly engaged in shared experiences with the team. Children are also engaged in the day to day running of the service through attendance at community meetings.

Parents are invited to join special events in the Touchstone community as well as being engaged on a regular basis through a parents group.

Children who have completed the program are invited to attend monthly leavers groups. These groups give them the opportunity to retain links with the service and with their peers, beyond the completion of the Day Program treatment and act as a platform to encourage and support the process of ongoing recovery.

In addition, ex patients of Touchstone may decide to become part of the CAMHS consumers group that provides input into the design and review of the service.

## Integration and Partnership

Touchstone commits to effective communication with other organisations and service providers, in order to advocate on behalf of children and families and facilitate access. The Touchstone case manager maintains contact with partner agencies and other service partners in order to be able to support families in exploring their treatment options and keeping them informed of potential recovery pathways.

As a part of the broader CAMHS directorate, there is regular liaison with Community and Acute CAMHS services to discuss treatment progress and plan for transition into and out of Touchstone. When children require admission to Inpatient Units or present to ED, Touchstone liaises directly with the ward's psychiatrist/allocated case manager and in ED with the PLNs or the consultant to ensure continuity of care and a return to the therapy program as soon as is safely possible. Touchstone liaises regularly with Tier 3 CAMHS and with education providers, via embedded School of Special Educational Needs: Medical and Mental Health (SSEN:MMH) teachers.

Shared care will occur with children who are medically managed by services such as, the Paediatric Consultation and Liaison Service, Gender Diversity Services and the Eating Disorders Service. Clear shared care agreements are drawn up to achieve clear and effective communication, in accordance with the [CAMHS Shared Care Policy](#).



### Related CAHS internal policies, procedures and guidelines

[CAMHS Access Policy](#)

[CAMHS Medical Record Management Procedure](#)

[CAMHS Patient Rights under the Mental Health Act Policy](#)

[CAMHS Shared Care Guideline](#)

### References and related external legislation, policies, and guidelines

[UN Convention on the Rights of the Child](#)

This document can be made available in alternative formats on request.

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|-------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|-------------------|-------------|
| Document Owner:                                                                                                                                                                                                                                                                                       | Director and Head of Department Specialised CAMHS                                                                                                                                |                   |             |
| Reviewer / Team:                                                                                                                                                                                                                                                                                      | Touchstone CAMHS Head of Service and Service Manager                                                                                                                             |                   |             |
| Date First Issued:                                                                                                                                                                                                                                                                                    | 29/10/2021                                                                                                                                                                       | Last Reviewed:    | 29/10/2021  |
| Amendment Dates:                                                                                                                                                                                                                                                                                      |                                                                                                                                                                                  | Next Review Date: | 29/06/2026* |
| Approved by:                                                                                                                                                                                                                                                                                          | Director and Head of Department Specialised CAMHS                                                                                                                                | Date:             | 29/10/2021  |
| Endorsed by:                                                                                                                                                                                                                                                                                          | CAMHS Oversight Committee Safe Systems and Practice                                                                                                                              | Date:             | 29/10/2021  |
| Standards Applicable:                                                                                                                                                                                                                                                                                 | NSQHS Standards: <br>NSMHS: 10.2, 10.3<br>Child Safe Standards: 1, 2, 3, 4, 5, 6, 7, 8, 9, 10 |                   |             |
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\*Note: CAMHS Leadership Committee approved a 20-month extension of the review date to 29/06/2026.



## Appendix 1: Day Program Care Pathway Flowchart

